MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY  VOLUME 15 OF 28

# MFT 715

# Family Systems Theory

15 teaching weeks  3 practicals  7 figures

SEMESTER TWO

1.71 units  Prerequisite: MFT 702

> This is the theoretical anchor of the programme.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.


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# MFT 715 Family Systems Theory

**MSc Marriage and Family Therapy** Semester Two 1.71 Units 15 teaching weeks Prerequisite: MFT 702

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.


### How to cite this volume

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### A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

# The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

| Code | Course | Semester | Units |
|---|---|---|---|
| MFT 701 | Fundamentals of Marriage and Family Therapy | One | 1.71 |
| MFT 702 | Theories of Family Therapy | One | 1.71 |
| MFT 703 | Human Development Across the Lifespan | One | 1.71 |
| MFT 704 | Ethical and Legal Issues in Therapy | One | 1.71 |
| MFT 705 | Research Methods in Marriage and Family Therapy | One | 1.71 |
| MFT 706 | Clinical Practice and Techniques | One | 1.71 |
| MFT 707 | Assessment and Diagnosis in Family Therapy | One | 1.71 |
| MFT 708 | Cultural Competence in Therapy | One | 1.71 |
| MFT 709 | Group Therapy Techniques | Two | 1.71 |
| MFT 710 | Couples Therapy | Two | 1.71 |
| MFT 711 | Child and Adolescent Therapy | Two | 1.71 |
| MFT 712 | Crisis Intervention Strategies | Two | 1.71 |
| MFT 713 | Substance Abuse and Family Dynamics | Two | 1.71 |
| MFT 714 | Trauma-Informed Care | Two | 1.71 |
| **MFT 715** | **Family Systems Theory** | Two | 1.71 |
| MFT 716 | Communication Skills in Therapy | Three | 1.71 |
| MFT 717 | Psychopathology in Families | Three | 1.71 |
| MFT 718 | Intimacy and Relationship Dynamics | Three | 1.71 |
| MFT 719 | Family Life Cycle and Transitions | Three | 1.71 |
| MFT 720 | Supervision and Consultation in Therapy | Three | 1.71 |
| MFT 721 | Research Methodology | Three | 1.71 |
| MFT 722 | Professional Development and Self-Care | Electives | 1.71 |
| MFT 723 | Special Populations in Therapy | Electives | 1.71 |
| MFT 724 | Family Law and Advocacy | Electives | 1.71 |
| MFT 725 | Integrative Approaches in Therapy | Electives | 1.71 |
| MFT 726 | Neurobiology of Relationships | Electives | 1.71 |
| MFT 727 | Play Therapy Techniques | Electives | 1.71 |
| MFT 770 | Master Thesis | Three | 1.71 |

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**Course leader's summary.** This is the theoretical anchor of the programme. Where MFT 702 surveys schools, this course interrogates the theory beneath them: cybernetics, general systems thinking, structure, communication, constructionism and their critics. You will finish able to explain precisely what you mean when you say a family is "a system".

## How to use this book

The word "system" is used loosely in clinical settings. This course tightens it. Each week takes one theoretical tradition, states its claims precisely, applies it to a case, and then asks what it cannot explain. You are expected to leave with arguments, not slogans.

## 1. Course description

Family systems theory is not one theory but a family of theories sharing a commitment: **the unit of analysis is the relationship, and the whole is more than the sum of its parts**. The course traces that commitment from von Bertalanffy's general systems theory and the Macy conferences on cybernetics, through the Palo Alto communication school, Minuchin's structuralism, Bowen's multigenerational theory, and the Milan group, to social constructionism, critical and feminist theory, and contemporary complexity and network perspectives.

## 2. Learning outcomes

| # | Outcome | Assessed |
|---|---|---|
| LO1 | Explain the core propositions of general systems theory and cybernetics | Exam |
| LO2 | Analyse a family using at least four theoretical traditions | Case analysis |
| LO3 | Critique each tradition, including its cultural and political assumptions | Essay |
| LO4 | Distinguish first- and second-order cybernetics and demonstrate reflexivity | Viva |
| LO5 | Apply constructionist ideas to meaning-making in a family | Case analysis |
| LO6 | Evaluate the evidence for systemic propositions | Exam |
| LO7 | Construct a coherent theoretical position of your own | Position paper |

## 3. Assessment

| Component | Weight | Due |
|---|---|---|
| Theoretical analysis of a case using four traditions (2,500 words) | 35% | Week 9 |
| Essay: critique of a systemic concept (2,000 words) | 25% | Week 13 |
| Examination | 25% | Week 16 |

| Component | Weight | Due |
|---|---|---|
| Position paper and viva (1,000 words) | 15% | Week 15 |

## 4. The fifteen-week plan

| Week | Topic | Practical |
|---|---|---|
| 1 | The systemic turn: what it was and what it answered | Compare two accounts of one case |
| 2 | General systems theory: wholes, boundaries, equifinality | Systems vocabulary drill |
| 3 | Cybernetics I: feedback, control, homeostasis | Feedback mapping |
| 4 | Cybernetics II: first and second order; reflexivity | Reflexivity exercise |
| 5 | Communication theory: axioms, punctuation, paradox | Transcript analysis |
| 6 | Structural theory: organisation, hierarchy, boundaries | Structural mapping |
| 7 | Multigenerational theory: anxiety, triangles, differentiation | Triangle mapping |
| 8 | Milan and meaning: beliefs, premises, circularity | Circular questioning |
| 9 | **Theoretical analysis due**; constructivism and social constructionism | Meaning-making interview |
| 10 | Postmodern and narrative theory: knowledge and power | Texts and stories exercise |
| 11 | Critical, feminist and anti-racist systems theory | Power audit of a model |
| 12 | Attachment and neuroscience meet systems theory | Integration diagram |
| 13 | **Essay due**; critiques and limitations of systems thinking | Debate |
| 14 | Complexity, networks and the contemporary field | Network map of a family |
| 15 | Your theoretical position | Viva |

## Week 1 The systemic turn

**What changed.** Before the systemic turn, the family was understood as the environment of an individual patient. After it, the family became the unit of treatment, and individual symptoms became functions of relationship patterns.

**What it answered.** Three problems the individual model struggled with: Why does a symptom appear in a particular child? Why does the family resist change (homeostasis)? Why does the same individual behave differently in different relationships?

**What it risked.** Dissolving the person into the system; treating "the family" as a homogeneous actor; ignoring power, gender, class and history; and producing descriptions that could not be falsified.

**PRACTICAL**

Take one case. Write it twice: once as an individual formulation (as a CBT or psychodynamic clinician would) and once as a systemic one. Then write a paragraph on what each version makes visible and invisible.

## Week 2 General systems theory

**Von Bertalanffy's propositions**, translated for clinicians:

| Proposition | Clinical meaning |
|---|---|
| A system is a set of elements in interaction | The family is defined by its interactions, not its members |
| The whole is more than the sum of its parts | The family has properties no member has alone |
| Systems are open or closed to their environment | A family that refuses outside contact becomes closed and rigid |
| Systems have boundaries | Who is inside, who is outside, and how permeable |
| Systems are hierarchical | Subsystems nested within larger systems |
| Equifinality | Different starting points can reach the same end state |
| Equipotentiality | Similar starting points can reach different end states |
| Negative entropy | Systems import energy to counter disorder (information, support) |

**Equifinality and equipotentiality are the most useful and most neglected.** Equifinality explains why two very different families produce the same symptom; equipotentiality explains why two similar families produce very different outcomes. Together they demolish simple causal stories.

**Example.** Two children are referred for school refusal: one in a family with high conflict and one in a family with high achievement pressure. Equifinality explains the identical presentation; equipotentiality explains why one family recovers in four sessions and the other takes two years.

## Week 3 Cybernetics I

**Feedback.** A system regulates itself by monitoring its own output and adjusting its input.

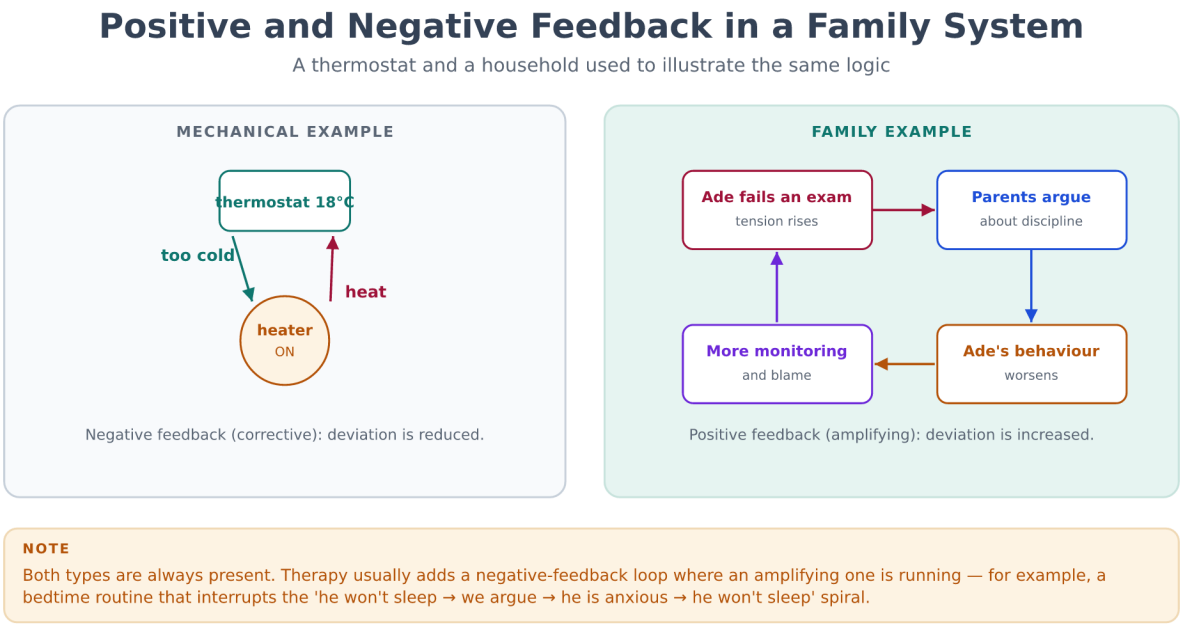



Figure 3.1 Negative and positive feedback in a machine and in a family

- **Negative feedback** corrects deviation: the thermostat, the bedtime routine, the family that de-escalates when voices rise.
- **Positive feedback** amplifies deviation: the microphone screech, the escalation, the panic spiral.

**Homeostasis.** The system's tendency to maintain its organisation. **Morphostasis** maintains; **morphogenesis** changes. A family can be stably unhappy: the symptom is one of the mechanisms that holds the system together.

**The clinical insight.** Attempts to change a system are themselves inputs. If the family's equilibrium depends on the symptom, your intervention will produce a counter-move. Predicting the counter-move and sometimes prescribing it is the strategic tradition's contribution.

**Calibration example.** A family's daughter has "tantrums" that end a 45-minute argument between her parents. The tantrum is negative feedback for the marital system (it restores calm) and positive feedback for the child (it works). Two feedback systems, one event.

> **PRACTICAL**
> Map both feedback loops in a disguised case: one for each "layer" (the couple system and the child's behaviour). Show how the same event serves both.

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## Week 4 Cybernetics II: first and second order

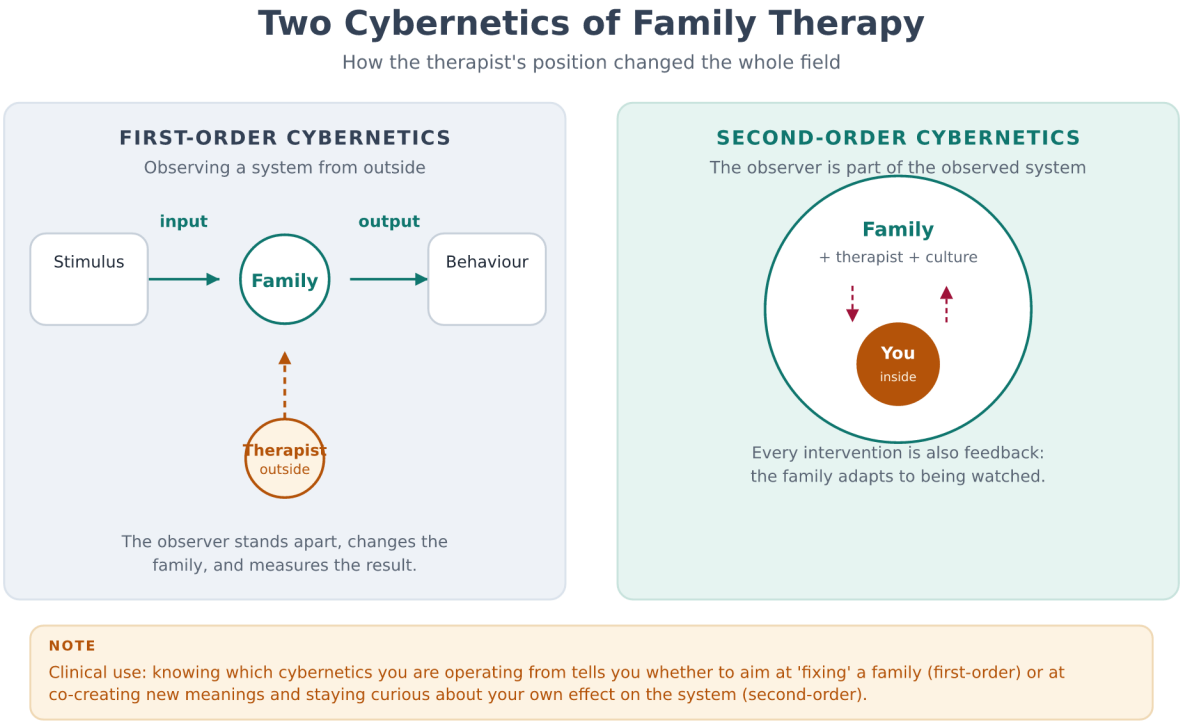



Figure 4.1 First- and second-order cybernetics: where the therapist stands

**First-order cybernetics.** The observer is outside the system; interventions are inputs; the aim is control of the system's output. This produced the heroic era: directive, strategic, clever.

**Second-order cybernetics.** The observer is part of the observed system; distinctions are drawn by an observer; knowledge is provisional. This produced curiosity, the reflecting team, and the therapist's accountability for their own effect.

**The clinical consequences.**

| First order | Second order |
|---|---|
| "What intervention will fix this family?" | "What am I noticing, and what am I blind to?" |
| The family is the object of change | The family and the therapist co-construct change |
| Neutrality is possible | Position is unavoidable; transparency is required |
| Expertise is the therapist's | Expertise is distributed |

**Reflexivity is not self-indulgence.** It is the discipline of accounting for your own contribution to the clinical picture: which family member you find easier to like, which story you believed first, which data you stopped collecting once you had a hypothesis.

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## Week 5 Communication theory




Figure 5.1 Watzlawick's five axioms of communication

**The five axioms** (Watzlawick, Beavin & Jackson).

1. One cannot not communicate.
2. Every message has a content and a relationship level.
3. Communication is punctuated into sequences by participants.
4. Messages are digital (words) and analogical (analogue).
5. Relationships are symmetrical or complementary.

**Double bind hypothesis** (Bateson): a person receives contradictory messages at different levels, cannot comment on the contradiction, and cannot leave the relationship. The hypothesis was weakened as an explanation of schizophrenia, but its clinical descendant recognising contradictory, un-commentable communications in families remains useful.

**Critique.** Communication theory could become mechanistic (families as machines exchanging messages) and could pathologise families of people with psychosis. It also tended to describe "systems" without the power differences that determine who gets to define what is communicated.

> **PRACTICAL**
> Transcript analysis: take ten lines of a couple's argument and identify punctuation, relationship-level messages, and symmetrical/complementary moves.

# Week 6 Structural theory

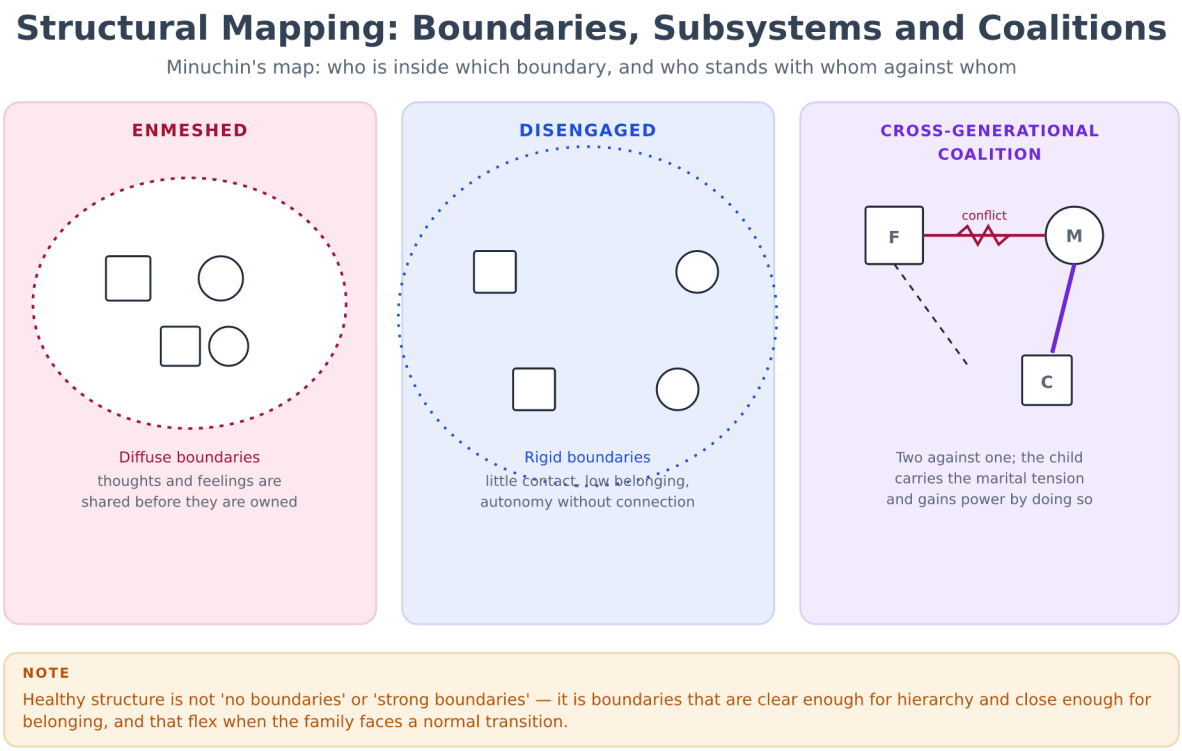



Figure 6.1 Structural mapping: enmeshed, disengaged and cross-generational coalition

**Core propositions** (Minuchin).

1. The family's structure is the invisible set of rules that govern transactions.
2. The structure organises the family into subsystems (spousal, parental, sibling) separated by boundaries.
3. Symptoms are best understood as the family's response to its structure, and sometimes as the only way the structure can remain stable.
4. Change in structure produces change in experience.

**The evaluation criteria for a structure.** Boundaries must be clear (not diffuse, not rigid); hierarchy must be appropriate to stage and culture; and subsystems must have enough autonomy to function and enough connection to belong.

**Power and structure.** Structural theory was criticised for assuming a normative hierarchy (executive parents, subordinate children) and for not asking how that hierarchy is upheld. Contemporary structural practice inserts the questions: whose authority is legitimate in this family, and who bears the cost of it?

# Week 7 Multigenerational theory

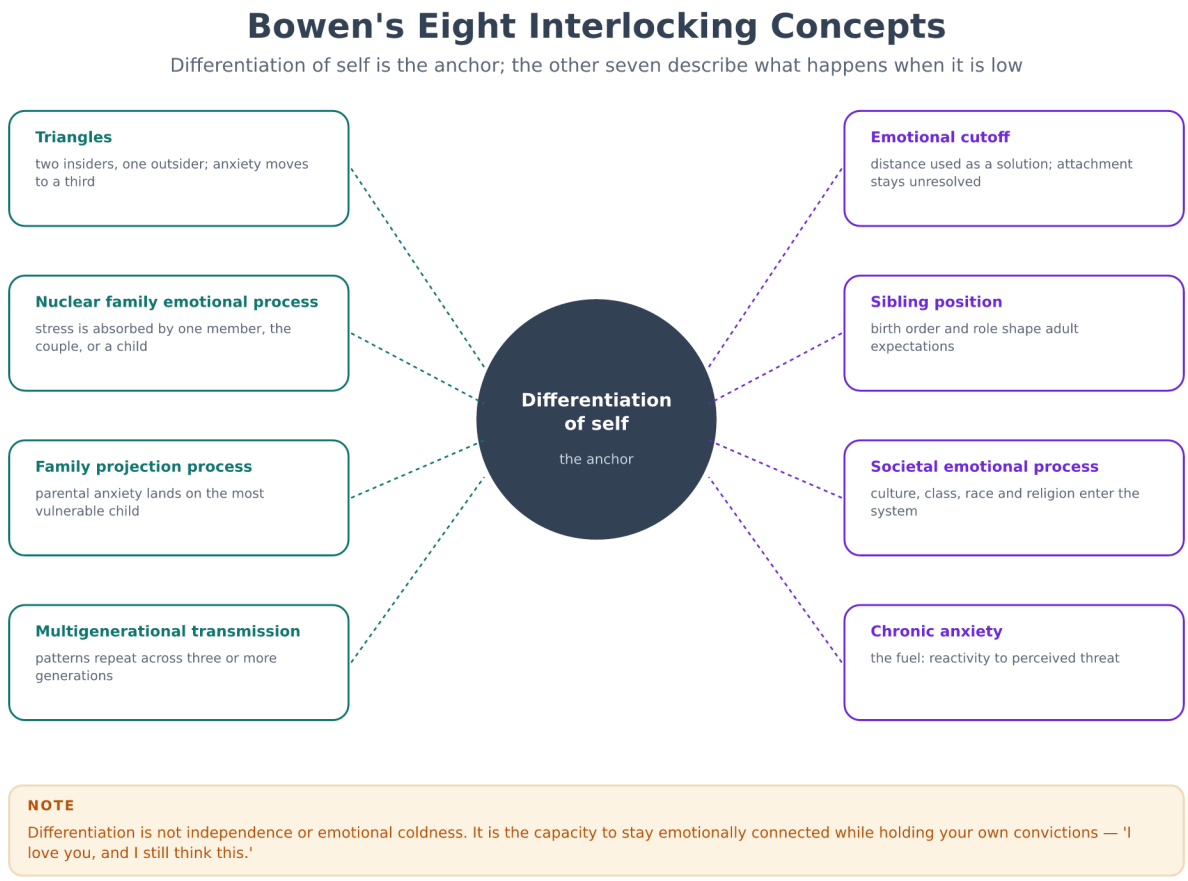



Figure 7.1 Bowen's eight interlocking concepts

**Core propositions** (Bowen).

1. The family is an emotional unit; anxiety is its most important variable.
2. Differentiation of self is the capacity to remain connected while thinking independently.
3. Anxiety is managed in one of four ways: conflict, distance, projection to a child, or dysfunction in one member.
4. Triangles distribute anxiety and stabilise dyads.
5. Patterns are transmitted across generations; symptomatic children are often the most triangulated.

**The theory's strength** is time: it offers an account of why now, by locating the present pattern in several generations of adaptation to war, migration, loss and religion.

**The theory's weakness** is its insufficient account of power, material constraint and culture. A woman cannot always "differentiate" out of an arranged marriage; a family cannot always "stay out of triangles" when migration has severed the network.

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## Week 8 Milan and meaning

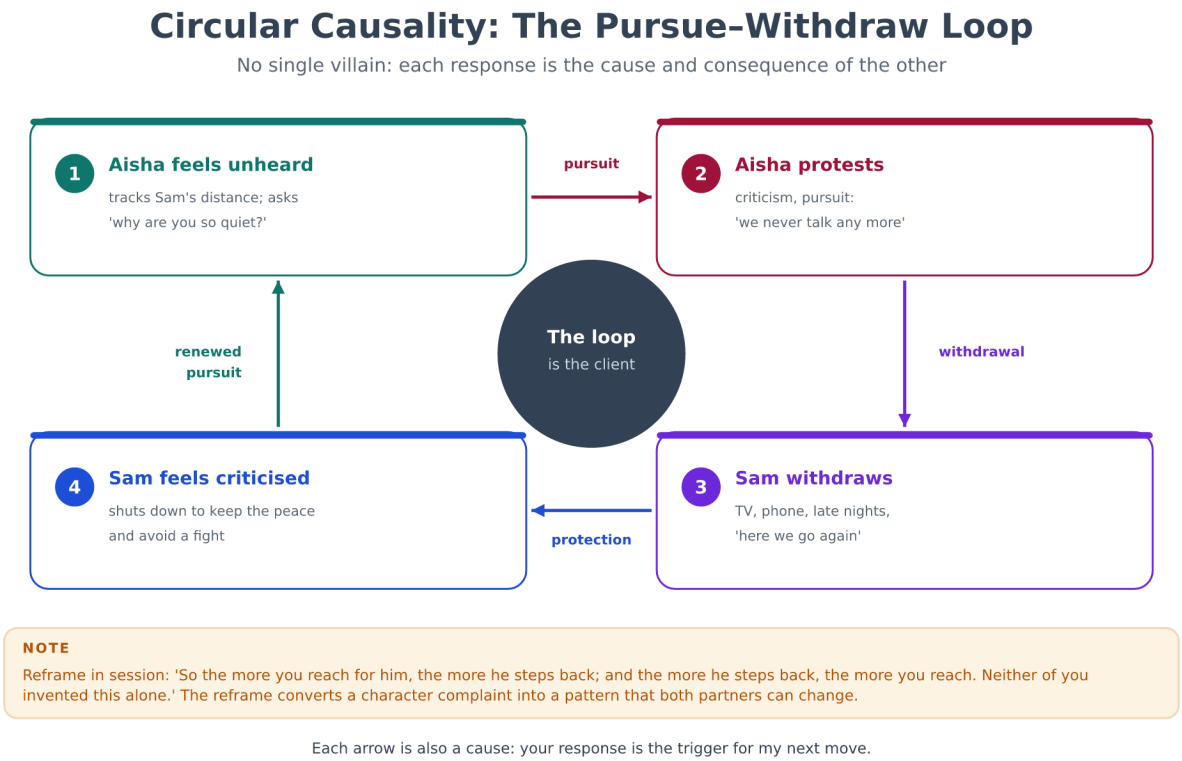



Figure 8.1 Circular causality in the consulting room

**Core propositions** (Palazzoli, Boscolo, Cecchin, Prata).

1. The family is a system of meanings (premises) that organise behaviour.
2. The therapist forms hypotheses and tests them with circular questions.
3. Neutrality is a stance of curiosity towards all members, not indifference.
4. Positive connotation frames all behaviour as serving the system's coherence.
5. Interventions disturb the system by changing premises, not by demanding behaviour change.

**The evolution.** Classical Milan used teams, rituals and directives; later Milan (and the reflecting team) abandoned directive intervention and emphasised multiple perspectives and the therapist's own openness.

**Critique.** Neutrality became indefensible in cases of violence and abuse; circular questioning can become an elegant evasion; and the intellectual register excluded many families.

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## Week 9 Constructivism and social constructionism

**Constructivism** (Maturana, von Glasersfeld): each person constructs a model of reality from their own experience; we cannot access an objective reality directly. Clinically: take each family member's account seriously, and stop trying to establish "the truth".

**Social constructionism** (Gergen, Shotter): meanings are produced between people, in language and in culture, and change with context. Clinically: the problem, the roles and the identities are made in talk, and so they can be made differently.

**The distinction matters.** Constructivism risks relativism and individualism (everything is "just your view"); social constructionism locates meaning in relationships and power, which makes it compatible with the critical and cultural traditions.

**Implication for practice.** The therapist's formulation is one description among several. Offering it humbly "here is a story I am beginning to see; how does it look to you?" is both a constructionist technique and a mark of respect.

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## Week 10 Narrative and postmodern theory

**Narrative** (White & Epston). Stories organise experience; dominant stories make alternatives invisible; the task is to externalise the problem and thicken a preferred story.

**Postmodern influences.** Foucault on discourse and power-knowledge; Derrida on deconstruction. The clinical translation: question the "official" account, ask who benefits from it, and look for what it silences.

**The reflexivity of narrative therapy** is unusual: the therapist is "decentred but influential", and the client is the author. This is an ethical stance as much as a technique.

**Critique.** The linguistic emphasis can under-serve families with limited literacy or in acute crisis; the evidence base, while growing, is smaller than the cognitive-behavioural tradition's.

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## Week 11 Critical, feminist and anti-racist systems theory

**The feminist critique** (Hare-Mustin 1978; Goldner; Walters) made four claims that changed the field:

1. "Circular" descriptions conceal power asymmetries.
2. "Neutrality" in the face of violence colludes with the perpetrator.
3. The gendered division of labour, not "the family system", is often the object of the wife's complaint.
4. Models that prize individuation encode a male norm.

**Anti-racist and postcolonial theory** extend the critique: whose family structures are treated as healthy? Which knowledge counts as evidence? How do services reproduce the state's relationship to marginalised families?

**Clinical translation.**

- Ask about power explicitly, in every assessment.

- Never use systemic language to make a victim responsible for the violence she receives.
- Place race, migration, class and religion inside the system, not outside it.

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## Week 12 Attachment and neuroscience in systems theory

**Two imports that have reshaped the field.**

1. **Attachment theory** gives systemic thinking a developmental mechanism: the couple and parent child systems are organised by attachment strategies, and those strategies are formed in relationships.
2. **Neuroscience** gives systemic thinking a physiology: co-regulation, the window of tolerance, mirroring and stress physiology explain how relationships change bodies.

**The integration.** "Systems" are not abstract patterns; they are embodied patterns between nervous systems. A couple's pursue withdraw cycle is simultaneously a communication pattern, an attachment strategy, and a sequence of autonomic states.

**Critique of the imports.** Neuroscience is often oversold in clinical texts; "mirror neurons" and "the right brain" are frequently invoked without evidence. Use physiology where it is well established (stress, regulation, sleep, trauma) and keep the metaphors labelled as metaphors.

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## Week 13 Critiques and limitations

| Critique | Substance | Response |
|---|---|---|
| The system has no agent | "The family did it" hides who actually did what | Locate action; ask who decided |
| Power is under-theorised | Systemic descriptions can neutralise abuse | Explicit power analysis |
| Culture is externalised | "The system" was a Western two-parent family | Cultural and critical turns |
| Evidence is thin | Many systems concepts are unfalsifiable | Operationalise and test |
| The therapist disappears | Neutral observer stance | Second-order reflexivity |
| Complexity obscures action | Endless circular description delays intervention | Use the simplest map that guides action |

**The mature position.** Systems theory is a disciplinary sensibility, not a complete theory. It tells you where to look (between people, across generations, in context) and what to resist (single villains and simple causes). It does not, by itself, tell you what to do.

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## Week 14 Complexity, networks and the contemporary field

**Complexity theory.** Families are non-linear, self-organising systems: small differences can produce large divergence; cause and effect are not proportional; and feedback loops are the rule. This fits clinical experience better than linear models, but it resists prediction.

**Network thinking.** Contemporary research treats families as networks of relationships, with density, centrality and clustering measurable. Who is central? Who is isolated? Who is the only link between two clusters? This is a formalised genogram, and it is improving our understanding of family structure and resilience.

**Ecological models.** Bronfenbrenner's nested systems (Figure 7 in MFT 701) place the family inside institutions, culture and time a corrective to the field's tendency to treat the family as self-contained.

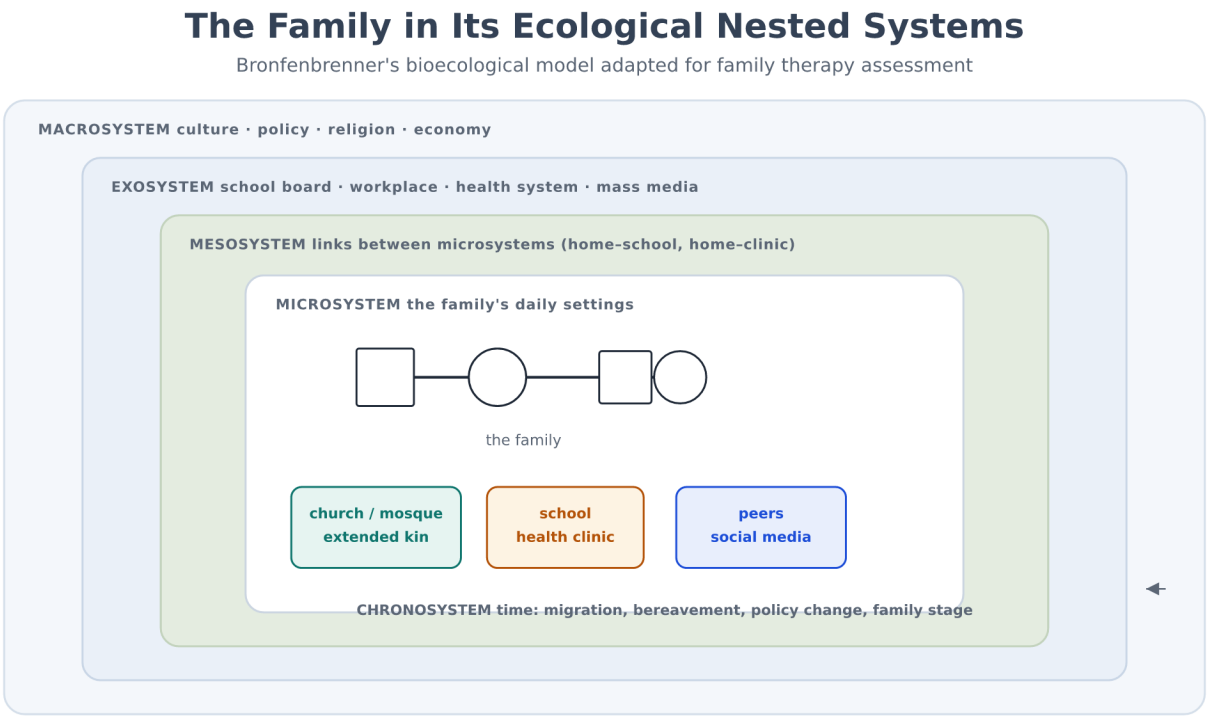



Figure 14.1 The family inside nested ecological systems

## Week 15 Your theoretical position

Write 1,000 words and defend it in ten minutes:

1. What is the unit of analysis you will work with, and why?
2. What do you believe about causation in families?

3. What is the therapist's position, and how will you manage your own power?
4. Which systemic concept do you find least defensible, and what do you use in its place?
5. What would make you abandon your position?

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## Glossary

**Autopoiesis** the self-producing organisation of living systems. **Boundary** the rules governing participation and exchange between systems. **Circularity** reciprocal causation within a system. **Cybernetics** the study of control and communication in systems. **Differentiation of self** see Bowen. **Equifinality** different paths leading to the same end state. **Equipotentiality** similar starting points leading to different end states. **Homeostasis / morphostasis** the maintenance of system organisation. **Morphogenesis** change in system organisation. **Punctuation** the marking of a sequence's beginning; the source of blame. **Reflexivity** the therapist's accountability for their own participation. **Second-order cybernetics** cybernetics of observing systems. **Structure** the repetitive patterns of transaction that organise a family.

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## Self-assessment

### Multiple choice

**1.** Equifinality means: (a) same cause, same outcome (b) different paths, same outcome (c) same paths, different outcomes (d) no causation

**2.** Homeostasis in a family refers to: (a) change (b) the maintenance of existing organisation (c) hierarchy (d) communication

**3.** Second-order cybernetics holds that the observer: (a) is outside the system (b) is part of the observed system (c) is irrelevant (d) is objective

**4.** "One cannot not communicate" is: (a) axiom 1 (b) axiom 3 (c) axiom 5 (d) a double bind

**5.** Punctuation is clinically important because it: (a) describes memory (b) assigns blame for a sequence (c) measures cohesion (d) is a diagnostic criterion

**6.** The feminist critique of family therapy argued that: (a) systems are too complex (b) circular descriptions conceal power asymmetries (c) hierarchy is unnecessary (d) therapy should be individual

**7.** Positive connotation is associated with: (a) structural theory (b) Milan (c) Bowen (d) behavioural therapy

**8.** A closed system in family terms is one that: (a) has clear boundaries (b) has limited exchange with its environment and becomes rigid (c) is disengaged (d) is enmeshed

**9.** Social constructionism locates meaning in: (a) the individual brain (b) relationships, language and culture (c) genetics (d) behaviour

**10.** The most defensible use of systems theory is as: (a) a complete predictive model (b) a disciplinary sensibility that shapes where you look (c) a diagnosis (d) a legal framework

### Answer key

1 b 2 b 3 b 4 a 5 b 6 b 7 b 8 b 9 b 10 b

### Short answer

1. Define equifinality and equipotentiality and give one clinical example of each.
2. Distinguish negative and positive feedback with a family example.
3. What are the three criteria for evaluating a family's structure?
4. State the feminist critique of "circular" formulations in four sentences.
5. What is reflexivity, and why is it a clinical skill?

### Essay (choose one, 2,000 words)

1. "Systems theory tells you where to look but not what to do." Discuss.
2. Critically evaluate the concept of homeostasis in families where violence is present.
3. Compare Bowen's and Minuchin's accounts of how problems develop, and assess which is better supported.

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## Further reading


- von Bertalanffy, L. (1968). General System Theory. Braziller.
- Watzlawick, P., Beavin, J., & Jackson, D. (1967). Pragmatics of Human Communication. Norton.
- Minuchin, S. (1974). Families and Family Therapy. Harvard University Press.
- Bowen, M. (1978). Family Therapy in Clinical Practice. Aronson.
- Becvar, D. S., & Becvar, R. J. (2018). Family Therapy: A Systemic Integration (9th ed.). Pearson.
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- Hare-Mustin, R. T. (1978). A feminist approach to family therapy. Family Process, 17(2), 181 194.


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